SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Oral etoposide · Days 1–3 twice daily

Extensive SCLC · Days 1–3 twice daily · NCCP 00388 · version 4 · 21-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
This treatment aims to slow or shrink extensive small-cell lung cancer when intravenous or combination chemotherapy is unsuitable. Benefit varies; your team explains the aim for you.

**Schedule**
Take your prescribed etoposide capsules twice daily on Days 1–3. Days 4–21 have no planned etoposide. The source cycle is 21 days; continue only while helping and tolerable, after team review.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Etoposide capsules**
TOPOISOMERASE-II INHIBITOR CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Fatigue and anaemia
- ☐ Nausea, vomiting, diarrhoea and mouth soreness
- ☐ Hair loss
- ☐ Allergic reactions
- ☐ Fertility and pregnancy effects
- ☐ Rare secondary leukaemia
- ☐ Rare serious skin reactions

Discuss treatment aim, alternatives including symptom-focused care, common and serious risks, fertility, possible delays or stopping, and what matters most to you.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00388 v4.

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